

Pearl Hospice LLC: company synopsis

Licensed, Medicare-certified hospice serving Cook, DuPage, Kane, Lake and Will counties, Illinois. Offered for sale with immediate transfer of operations.

Pearl Hospice is a freestanding, Medicare-certified hospice with a clean Illinois license through July 2027, a provider agreement dating from 2019, and a five-county Chicago-area service territory. The owner has decided to exit healthcare operations and is seeking a buyer who will take over day-to-day operations and regulatory responsibility now, not at the end of a multi-year management term. In return the seller is prepared to price the business for a quick, clean closing.

CCN 141701
Medicare certified Sep 13, 2019; Palmetto GBA JM

Jul 31, 2027
IDPH Comprehensive Hospice license 2003258 expires

5 counties
Cook, DuPage, Kane, Lake, Will; Medicare and Medicaid

\$806K
2025 gross receipts; 46 Medicare beneficiaries served

What is for sale

The operating hospice as a going concern: its license, Medicare provider agreement, name and goodwill, patient and referral relationships, policies and records, vendor contracts, office lease, equipment and technology. The seller will also entertain a membership-interest purchase of the LLC if the buyer prefers to keep the CCN inside the existing entity.

Item	Detail
Legal entity	Pearl Hospice LLC, Illinois, formed November 2017; dba Devoted Home Hospice (IDPH) and Pearl Palliative and Hospice (CMS)
Ownership	Single owner since the July 2022 change of ownership. The 36-month rule window (42 C.F.R. 424.550) closed in July 2025, so a buyer does not re-enroll as a new provider
Licensure	IDPH Comprehensive Hospice Agency, license 2003258, expires July 31, 2027. No accreditation; CMS deemed status is not required for hospice
Medicare	CCN 141701, NPI 1346754173, certified September 13, 2019, fiscal year end December 31
Location	2720 S River Rd, Suite 238, Des Plaines, IL 60018 (leased office)
Levels of care	Routine home care in patients' homes and contracted facilities. Inpatient and respite capacity through nursing-facility agreements (Mado Healthcare Uptown, RYZE at the Ridge, Prairie Manor)

Illinois market	No certificate-of-need requirement for hospice. The licensed CCN is the gating asset; a new entrant faces enrollment, initial survey and 12 to 18 months of ramp
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Operating profile

The agency built from a dormant shell in 2022 to an average daily census of about 11 in 2025, nearly all Medicare routine home care. Census has since contracted, and the current staff is sized for a smaller book. A buyer with an existing referral network or back office can restore volume quickly on the licensed platform.

Measure	2023	2024	2025	2026 to date
Gross receipts (Form 1065)	\$200,453	\$890,889	\$806,043	\$266,277 Medicare, through July
Medicare beneficiaries (unduplicated)			46	15 admissions
Routine home care days			4,084	
Average daily census			11.2	2 to 6
Net Medicare reimbursement (PS&R)			\$760,547	
Book net income (Form 1065)	\$34,505	\$168,588	-\$125,721	
W-2 payroll	\$17,006	\$40,577	\$213,900	about \$148,000 annualized

2025 expenses included about \$148,000 of management and consulting fees and \$265,000 of unitemized contract services. A buyer who absorbs administration into an existing platform removes most of that cost; on that basis 2025 would have produced positive earnings of roughly \$50,000 before any census recovery.

People and infrastructure in place

Clinical and administrative - Administrator (RN) and clinical supervisor (RN) on direct contract - Two staff RNs, one social worker, one chaplain, one hospice aide on W-2 payroll - Part-time medical director and alternate physician listed on the IDPH filing - Operations manager handling vendors, payroll and billing coordination	Contracts - DME benefit management: InvaServ / StateServ, three-year term from January 2026 - Pharmacy: Uvanta Pharmacy, Des Plaines, since 2022 - Nursing-facility room and board, GIP and respite: Mado Healthcare Uptown, RYZE at the Ridge; Prairie Manor by W-9
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- Therapy staffing: Strive Therapy and Archos Medical Staffing
- Eligibility evaluations: Absolute Medical Group (physician group)
- Music therapy: Greater Chicago Music Therapy

Most vendor agreements require consent to assign, which the seller will request as part of closing; a buyer with its own DME, pharmacy and therapy vendors may replace them instead.

Matters a buyer should plan for

The seller is disclosing these up front so that offers reflect them and diligence holds no surprises.

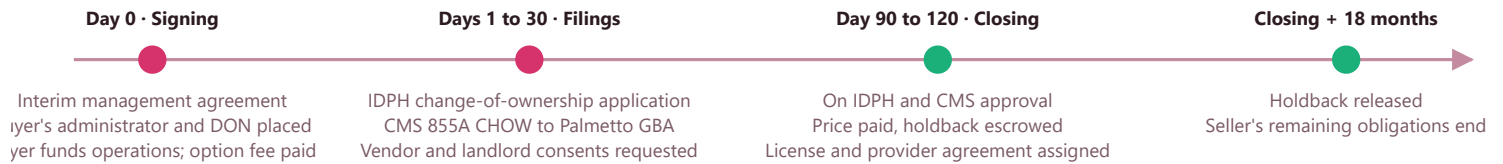
1. **Medicare prepayment review.** On June 10, 2026 the Midwest Unified Program Integrity Contractor (CoventBridge) placed the agency on prepayment medical review of all claims. Six additional documentation requests issued September 24, 2026 covering May through July 2026 service months (\$36,094 in charges) are due November 8, 2026. No reason or error rate was stated. The seller will fund and deliver the ADR responses and will carry the resulting liability for pre-closing dates of service.
2. **Census.** Average daily census fell from about 11 in 2025 to the low single digits in 2026. The licensed capacity, contracts and referral history remain; volume must be rebuilt.
3. **Staffing.** Current payroll is seven people. A buyer should plan to place its own administrator, director of nursing and volunteer and bereavement coordinators.
4. **Documentation to be completed.** The seller is assembling the CMS tie-in for the 2022 change of ownership, the operating agreement reconciling the ownership percentages across CMS, IDPH and tax filings, the medical director agreement, and current insurance certificates.
5. **Affiliated business.** The owner also operates a home health and staffing company (Devoted Healthcare) at the same address. Shared services will be separated at closing and the seller will sign a non-compete covering hospice in the five counties.

Proposed transaction: operations and liability transfer at signing

The seller's objective is to be out of hospice operations and regulatory responsibility as soon as the law allows. A conventional management-agreement-with-option, in which the seller holds the license for three years while a manager runs the business, does not meet that objective and will not be accepted. The structure the seller is offering moves control, cost and risk to the buyer in stages, each as early as the rules permit.

Buyer responsibility from Day 0: staffing, patient care operations, billing, cash funding, survey readiness, ADR responses for post-signing dates of service

Seller retains only what the law requires the licensee to keep until closing, plus pre-signing liabilities covered by the holdback



Illinois and CMS rules require the licensed owner to remain the provider of record until the change of ownership is approved. Everything that can move earlier, moves at signing.

Term	Seller's proposed position	Why
Form	Asset purchase of the hospice business, or membership-interest purchase of Pearl Hospice LLC at the buyer's election, preceded by an interim management agreement	Buyer chooses the structure that best preserves the CCN for its purposes
Interim management	From signing to closing. Buyer provides the administrator and director of nursing, runs operations, funds working capital, and indemnifies the seller for everything arising from post-signing operations. Fixed monthly fee at fair market value, not a profit share. IDPH and CMS notified of the management arrangement within 30 days	Transfers operational liability on Day 0 in a form that satisfies the anti-kickback safe harbor and the change-of-information rules
Term limit	Interim period capped at 180 days, extendable only for pending regulatory approval	The seller will not hold the license indefinitely for a buyer who has not closed
Price	Offers invited. Guidance: at or above \$100,000 for the licensed platform and goodwill, paid in cash at closing, with the option fee credited. Receivables excluded or purchased at an agreed discount with a collections true-up	Prices the asset for what it is today and gives the buyer the upside of rebuilding census
Option fee	\$25,000 to \$50,000 at signing, non-refundable except if a regulator bars the transfer	Compensates the seller for taking the business off the market during the interim period
Holdback	25% to 35% of price held in escrow for 18 months against Medicare overpayments, recoupments and ADR denials on pre-signing dates of service. The seller's exposure for those matters is capped at the holdback	Gives the buyer real protection while giving the seller a defined, finite liability instead of an open-ended indemnity

Pre-signing liabilities	Remain with the seller, subject to the cap above. The seller will prepare and submit the six pending ADR responses with buyer input	Clean line between the seller's history and the buyer's operation
Representations	Customary representations on licensure, enrollment, billing compliance, litigation and taxes, surviving 18 months. Seller to deliver the 2022 CHOW tie-in, operating agreement, medical director agreement and insurance certificates before closing	Buyer gets a documented platform; seller's obligations end with the survival period
Seller release	At closing: resignation from all officer, administrator and authorized-official roles; removal from bank, PECOS and IDPH records; buyer's assumption of the lease and assigned contracts; mutual release of post-signing claims	This is the seller's principal objective
Non-compete	Seller and affiliated entities: no hospice in the five counties for three years	Protects the goodwill the buyer is paying for
Exclusivity	During the interim period only	No multi-year lockup

Why a buyer should prefer this structure. The buyer controls operations from the first day and can start rebuilding census on its own referral network without waiting for regulatory approval. It acquires a licensed, enrolled Medicare hospice in a no-CON state for the cost of a shell, with the prepayment-review history ring-fenced behind a holdback, and it avoids the fraud-and-abuse exposure that a profit-share management arrangement carries for both sides.

Process

1. Sign the non-disclosure agreement and receive data-room access: 37 documents covering corporate records, licensure and enrollment, 2022 to 2025 tax returns and financial statements, payroll, vendor agreements and the 2026 prepayment-review correspondence.
2. Management call with the owner and administrator; site visit on request.
3. Indicative offer letter stating price, structure (asset or membership interest), option fee, holdback and timing.
4. Selected buyer: interim management agreement and definitive purchase agreement negotiated concurrently; target signing within 30 days of selection.

The seller intends to select a buyer in the fourth quarter of 2026 so that the interim management period begins before year end and the change of ownership is approved in the first quarter of 2027.

Medicare PS&R, payroll registers and the IDPH license renewal application; the seller makes no representation beyond those in a definitive agreement.